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Title: DNAR Policy

1. Purpose:

Assist health care professionals to make decisions on **Do-Not-Attempt-Resuscitation (DNAR)**.

This document is nationally endorsed as a legal framework and is the official DNAR policy in the Sultanate of Oman.

2. SCOPE: All Health Care Facilities in the Sultanate of Oman

3. Definition / Abbreviations:

For the purpose of this policy, the following definitions are to be followed:

3.1 Adult – Any person who is 18 years of age or older.

3.2 Advanced Directive (Written / Verbal) - is a document by which a person makes provision for health care decisions in the event that, in the future, he/she becomes unable to make those decisions. Such directive must not contradict Islamic Principles.

3.3 Appropriate Health Care Facility Authority - The Director General of the Health Care Facility or a senior clinician/ Head of the concerned department.

3.4 Capacity – The capability of understanding and appreciating the nature and consequences of a DNAR order and the medical decisions to reach an informed decision regarding such an order. The physician-in-charge makes this determination to a reasonable degree of medical certainty.

3.5 Cardiopulmonary Resuscitation (CPR) – Measures to restore cardiac function and/or to support ventilation in the event of a cardiac or respiratory arrest, such as manual chest compression, artificial airway intubation (including endotracheal intubation), artificial ventilator assistance (including mechanical ventilation), intravenous resuscitative medications, electrical defibrillation, cardiac pacing and open chest cardiac massage. DNAR order may cover all cardiopulmonary resuscitative measures or may be limited to specific procedures or equipment depending on the scope of the consent.

3.6 Clinical Physician Privilege on Withholding Information - When a Physician for good reason believes that disclosure of the truth to the patient would cause irreparable and grievous harm. Clinical privilege should not be a reason not to provide information pertaining to the patient or to the Surrogate Decision Maker.

resuscitative efforts.

3.8 Concurrent Care Concerns – A DNAR order only means that the patient is not to receive resuscitative measures in the event of cardiac arrest. Any other limits on medical interventions (such as “do not intubate” or “no blood products”) need separate orders. A patient’s Concurrent Care Concerns are defined as any other limits besides the DNAR order as well as attention to the patient’s symptomatic, psychosocial, and spiritual needs.

3.9 Concurring Physician - A physician selected to provide concurring opinion in the event of dispute.

A concurring physician should not be subordinate to the physician-in-charge; any other experienced physician.

3.10 Do-Not-Attempt-Resuscitation order (DNAR) – This is an order issued by the physician-in-charge that tells the medical team not to perform CPR on a patient in the event of cardiopulmonary arrest.

3.11 DNAR Status - The decision about the code whether to administer the CPR or not in the event of cardiopulmonary arrest.

3.12 CPR Futility - CPR is considered futile if the resuscitative efforts are not in the best interest of patient and deemed unlikely to restore adequate cardiac or respiratory function in the event of cardiac respiratory arrest.

3.13 Health Care Provider - means any physician, dentist, nurse, paramedic, psychologist, or other person providing medical, dental, nursing, psychological, or health care services of any other kind.

3.14 Incapacity - means the inability, because of physical or mental impairment, to appreciate the nature and implications of a health care decision; and/or to make an informed choice.

3.15 Junior Medical Staff - Qualified medical staff with privilege to practice clinical medicine, next to the physician-in-charge in the seniority hierarchy of the Health Care Facility and who is available at the Health Care Facility for the patient care.

3.16 Minor - Any person who is less than 18 years of age.

3.17 Physician-in-charge - Means physician, with privilege to practice clinical medicine in the Health Care Facility, under whom the patient was admitted and has primary responsibility for treatment or care of that patient.

3.18 Surrogate Decision maker (SDM) – A person eligible to make decisions regarding DNAR status on behalf of a patient in the absence of Advanced Directive when the patient is unable to make decisions. The surrogate should be from the following list (in order of priority as listed below). In the event of an individual higher in this hierarchy not being available, not willing or not competent to make a decision regarding DNAR status, the SDM should be one next from this hierarchy:

3.18.1 The spouse

3.18.2 A parent

3.18.3 Eldest available son or next in line

3.18.4 Brother

3.18.5 Closest relative(s) accompanying the patient.

3.18.7 The legally appointed decision maker

4. DNAR Policy:

4.1 Introduction

Changing concepts of health and disease have incited health personnel to think carefully before making decisions related to initiation, continuation and termination of life support of patients suffering from condition, which has a poor prognosis, or may result in a poor quality of life.

As with many medical decisions, deciding whether to resuscitate a patient involves careful consideration of the potential for clinical benefit as well as the patient’s preferences for intervention. The decision to forego resuscitation is often difficult because of a real or perceived conflict of the aforementioned considerations.

Consequently, building on “The National Committee of Bioethics (NBC) Fifth Forum” conducted on the 2nd of June, 2014 under the banner “Do Not Resuscitate (DNR): Searching for an overall Approach”, the need for a national DNR policy for all Health Care Facilities in the Sultanate was recognized and discussed in the National Bioethics Committee meetings. Beside the conclusive remarks that emerged from the Fifth NBC Forum, the “DO NOT RESUSCITATE POLICY- SQUH- POL 04 REVISION 18-5-2011 was seen as the most useful framework to build on formulating a National DNR Policy. A recent comprehensive publication by the British Medical Association, Resuscitation Council (UK) and Royal College of Nursing “Decisions Relating to Cardiopulmonary Resuscitation” 3rd edition, October 2014, was also an important resource highlighting current practice in other experienced health care system.

4.2 Principles

4.2.1 GENERAL: Since human life is a gift from God, there is a primary moral obligation to show reverence for that life at all times from its beginning until death. Any failure to show reverence for or to safeguard a patient’s life is an assault on the individual patient, on others involved, on the medical profession, on society and dereliction of the rights of God. The sanctity of human life is ordained in the Quran “anyone who has killed a fellow human except in lieu of murder or mischief on earth, it would be as he slew the whole mankind”. Consequently, when a decision not to resuscitate is under consideration, the following principles should be adhered to by all health care providers involved in the care of patients.

4.2.2 This policy is intended to apply to all patients in all Health Care Facilities and health care facilities in the Sultanate of Oman. It included adults of all ages and categories, children of all ages (Children DNAR Policy under review by central committee to be updated by end of 2016) and pregnant women, both inpatients and outpatients.

4.2.3 No physician, nurse, or other personnel or care provider should participate in assisted suicide or euthanasia. Euthanasia is an action or omission which of itself or by intention causes death in order that suffering may be eliminated. A physician may not encourage a patient to violate his or her moral obligations, help him or her to do so, or refuse a patient’s request for treatment or therapy that is obligatory.

4.2.4 Patients who are not citizens of Sultanate of Oman and considered as expatriates whose culture, values and preferences may not be similar to citizens of the Sultanate need to comprehend the National DNAR policy in advance. Existence of “Advanced directives”, legally valid “Will” and the hierarchy of SDM specific for such patients should be enquired, addressed appropriately after due consideration of such patient’s best interest and preference. Conflicts between implementation of this policy and patients preference should be resolved by referring to the appropriate authority.

4.2.5 The decision about resuscitation - the DNAR order - should remain distinct from other medical decisions. The physician-in-charge should ensure that DNAR is not construed as “nothing can be done” by other health care workers, that patient are not abandoned and health-care providers should maintain providing the following measures if these improve the quality of life:

4.2.5.1 Administration of oxygen, suction of the airway.

4.2.5.2 Positioning of patient for comfort

4.2.5.3 Consulting other health care providers for expert care.

4.2.5.4 Attending to nutritional and hydration needs.

4.2.5.5 Provision of antibiotics if that help to control or prevent symptoms.

4.2.5.6 Treat reversible medical conditions.

4.2.5.7 Ensure maintenance of patient’s urinary and bowel system patent.

4.2.5.8 Pain medications.

4.2.5.9 Emotional support.

4.2.5.10 Relieving anxiety.

4.2.5.11 Arranging for the necessary psychosocial and spiritual support.

4.2.5.12 Providing nursing care to support the goal of keeping the patient as comfortable as possible.

transfusion, blood sampling for laboratory studies and radiologic or other diagnostic imaging procedures.

Any controversial procedure eg Dialysis requiring inotropes for comfort will require 2 consultants to take the decision

4.2.6 A patient has the moral obligation to accept any treatment, medication or operation, which offers a reasonable hope of benefit without requiring excessive fortitude, e.g., without causing excessive pain or endurance to the patient herself/himself without undue influence from health care providers, family members or friends.

4.2.7 The patient has the right to refuse any treatments, medications, operations or any act of healthcare workers that aim to aid recovery or prevent further progression of her/his illness.

4.2.8 A Health Care Facility exists to diagnose and treat patients. The goal is always to provide the best available medical care to the patient. Consequently, for a Health Care Facility to purposely expedite death by omission or commission violates a fundamental principle of medicine (do no harm). Recognizing that not every illness can be cured, but that every patient must be cared for, a Health Care Facility cooperates with other facilities and services, as well as the patient's family, to deliver the best care possible to the patient.

4.2.9 When it is determined, that a patient is dying of a terminal condition with or without treatment, the patient has the right to request or refuse such treatment or therapy if it does not impact on others or on the society at large.

4.2.10 All patients should be encouraged to express in advance, when they are still in capacity, their preference regarding the initiation of CPR, especially those at risk of cardiopulmonary arrest. Categories of patients who are likely to need counseling are:

4.2.10.1 Patients with terminal conditions

4.2.10.2 Patients with poor quality of life as judged by themselves

4.2.10.3 Patients with severe and irreversible illnesses or disabling conditions

4.2.10.4 Patients with increased risks of cardiac arrest or respiratory arrest

4.2.12 CPR considered inappropriate when cardiac arrest occurs in patients with the following conditions:-

4.2.12.1 Irreversible end-stage cardiac, hepatic or pulmonary failure

4.2.12.2 Multiorgan dysfunction in patients with terminal conditions

4.2.12.3 Refractory shock with multiorgan dysfunction on maximum support

4.2.12.4 Vegetative states due to permanent neurological damage

4.2.12.5 Patients with documented brain death, except potential organs donors (See section on Brain Death annexed to this policy).

4.2.12.6 Patients with established signs of death on arrival to Health Care Facility

4.2.13 There shall be no discrimination based on age, sex and social status. DNAR decisions are determined by patient's health status.

4.2.14 Validity & Duration of DNAR Form:

A DNAR order issued in any institution that complies with this policy is a clinical decision of the doctor in-charge of the patient and the treating team. The patient or the Family / SDM must be fully informed but their consent is not required.

4.2.15 Validity Of DNAR Orders For Referred Patients From Another Health Care Facility:

Physician-in-charge should review the DNAR order issued from another Health Care Facility and re-affirm its validity when appropriate. The patient or the Family / SDM must be informed.

4.2.16 Conflict Resolution:

4.2.16.2 Where there is an unresolved conflict between the physician-in-charge and the patient or Family / SDM regarding the DNAR status, another physician (concurring physician) opinion is sought.

4.2.16.3 Where there is a conflict between the physician-in-charge and the concurring physician regarding DNAR status, the matter should be referred to the Health Care Facility Ethics Committee.

4.2.16.4 Where there is a conflict between a minor and parents, the matter should be referred to the Health Care Facility Ethics Committee. However in circumstances that need an immediate decision, it must be recognized by all that DNAR is a purely medical decision.

4.2.16.5 Where there is a conflict between an adult patient with capacity and family regarding DNAR status in the presence of the patient's advanced directive not to attempt resuscitation on him/her, patients' decision shall prevail. However in circumstances that need an immediate decision, it must be recognized by all that DNAR is a purely medical decision.

In addition, conflict of procedures of labeled DNAR patients should be resolved in similar steps.

4.2.17 Audit and Review

The adherence of this policy needs to be audited and reviewed periodically by the **National Bioethics Committee** every three years to address future changes in the concept of DNAR status and the values and preferences of consumer advocacy groups.

4.3 The Process of Decision Making

4.3.1 Every patient admitted to a Health Care Facility is presumed to have consented to CPR and the trained medical and paramedical employees of the Health Care Facility have professional and legal obligation to institute CPR in the event of cardio-pulmonary arrest unless a DNAR order is in place in a manner that is consistent with this policy.

4.3.2 Every patient is presumed to have capacity unless there has been a determination of lack of capacity (Incapacity) as determined to a reasonable degree of certainty by the physician-in-charge.

4.3.3 DNAR order is only offered when CPR will be of no benefit on the grounds of medical futility or poor quality of life (refer to 4.2.12).

4.3.4 Clinicians must avoid imposing their own personal value judgments and apply professional knowledge and judgments when deciding the quality of life, subsequent to a potentially successful CPR.

4.3.5 The physician-in-charge should take all efforts to gather the subjective and objective data that determine prognosis or survival before establishing medical futility as the basis for withholding CPR.

4.3.6 Knowledge of the probability of success with CPR may be used to determine its futility.

4.3.7 Junior medical staff and the nursing staff should be encouraged to contribute to the decision making process.

4.3.8 Judging "Quality of life" tempts prejudicial statements about patients with chronic illness or disability. There is substantial evidence that patients with chronic conditions often rate their quality of life much higher than might be assumed. However, there is probably consensus that patients in a permanently unconscious state possess a quality of life that few would accept. Therefore, CPR is usually considered as "futile" for patients in a persistent vegetative state. Clinical responsibility for final decision about withholding long life support treatment rests with the physician-In-Charge and the treating team.

4.3.9 DNAR Orders in the Perioperative Period and Potentially Reversible Causes of Cardiorespiratory Arrests

4.3.9.1 Unless there is a valid and applicable advance decision refusing CPR, a DNAR decision is not binding. The final decision regarding the application or not of the CPR in an emergency rests with the healthcare professionals responsible for managing the patient's immediate situation but healthcare professionals must be able to justify their decision, in particular, clinicians should be cautious of overriding a DNAR decision where the patients has expressed a clear wish not to receive attempted CPR.

contrary to DNAR decision for a readily reversible cause such as choking, or blocked tracheal / tracheostomy tube. In such circumstances CPR would be appropriate, while the reversible cause is treated, unless the person has made a valid refusal of the intervention in these particular circumstances. To avoid misunderstandings it may be helpful, whenever possible, to make clear to patients and those close to patients that DNAR decisions usually apply only in the context of an expected death or a sudden cardiorespiratory arrest and not to an unforeseen event such as a blocked airway.

4.3.9.2 The DNAR order of patients who need anesthesia should be re-evaluated by the senior anesthesiologist and surgeon in the presence of the physician-in-charge and patient or Family / SDM. It should be affirmed, clarified or revoked for the intra- operative period. It may be appropriate to suspend a decision not to attempt CPR temporarily during some procedures, if the procedure itself could precipitate a cardiorespiratory arrest, especially if there is a high probability that prompt treatment of the arrest may be effective. For example, cardiac catheterization, pacemaker insertion, or surgical operations that may occasionally trigger cardiorespiratory arrest.

General or regional anesthesia may cause cardiovascular or respiratory instability that requires supportive treatment, which may include CPR. Many routine interventions used during anesthesia (for example tracheal intubation, mechanical ventilation or injection of vasoactive drugs) may also be regarded as resuscitative measures. If a patient wants a DNAR decision to remain valid during such procedure or treatment that carries some risk of cardiorespiratory arrest this may increase the mortality risk of the procedure or treatment. As an extreme example, some cardiac surgical procedures require induction of cardiac arrest as a necessary part of the procedure, so treatment could not be completed successfully without reversal of that arrest by defibrillation. If a clinician believes that a procedure or treatment would not be successful or would be unacceptably hazardous with the DNAR decision still in place, it would be reasonable not to proceed.

4.3.9.3 The anesthesiologist should document the agreed management option status for the perioperative period and should communicate the decision to all healthcare staff managing the patient in the operating theater and recovery areas. The DNAR decision should be reinstated once the effect of anesthesia and its medications wears off unless in exceptional circumstances where there is no agreement on which DNAR decision option should be adopted, the decision should be referred to the Health Care Facility ethics committee.

4.3.10 Patients Advanced Directives and DNAR Consent

4.3.10.1 DNAR should be an informed clinical decision like any other major diagnostic or therapeutic clinical decisions; NO CONSENT IS REQUIRED. Before such DNAR order is declared, the clinician-in-charge should make sure and document evidence that resuscitation is futile beyond doubt and to be supported with objective clinical or laboratory criteria or data.

4.3.10.2 The physician-in-charge should give to the patient or Family / SDM “if patient lack capacity” all relevant information including the objective data, based on which the DNAR status is decided upon. The physician will document these discussions in the medical record, and a second staff member will testify to his or her participation as a witness.

4.3.10.3 Patients who are pregnant and potential candidates for DNAR should be referred to the ethical committee to decide on the DNAR status.

4.3.10.4 In the event of uncertainty about DNAR status, resuscitative measures should be initiated.

4.4 Family Role

4.4.1 If CPR is judged medically futile, the physician is under no obligation to provide CPR but he/she must communicate the end-of-life care including the DNAR status to the family with detailed information of the condition and an unambiguous understanding of the clinical situation. Most Families will agree with the decision taken of the DNAR order.

Occasionally patients may object to informing the family then such wish should be documented in the patient’s record.

4.4.2 If the physician-in-charge judges that the patient lack capacity then the concerned family members should be informed.

4.5 Documentation

4.5.1 If a DNAR order is not available at the time of cardio-respiratory arrest and the DNAR status of the patient is unknown, health care workers are expected to administer CPR, consistent with clause 4.3.1 of this Policy.

4.5.3 It is the responsibility of the physician-in-charge to comply with the principles of this policy while issuing a verbal order to other health care workers with respect to the DNAR status.

4.5.4 In Emergency situations ONLY, Any DNAR order that is signed by a registrar and above is valid after informing Physician-In-Charge and documenting the discussion that took place by phone.

4.5.5 A DNAR order should be precise and unambiguously documented in the medical record. Terminologies such as "for-resuscitation but not for ventilation" "resuscitate but do not ventilate", etc. are confusing and should be avoided. In addition DNAR icon and form should be filled in the Al Shifa medical system by pressing on the DNR icon on the top right hand corner of the screen.

4.5.6 The above entries must be made in the medical record and should include the following:

4.5.6.1 The diagnosis

4.5.6.2 Prognosis

4.5.6.3 Recommendations of the treatment team or consultants (with documentation of their names and role)

4.5.6.4 The reasons for the DNAR status

4.5.6.5 Type of the DNAR order including any specific instructions

4.5.6.6 Discussion and Involvement of the patient, family or SDM

4.5.6.7 Disagreements, if any and the methods taken to resolve the conflict

4.5.7 The DNAR documented should reflect the DNAR status including specific instructions which are documented in the medical record.

4.5.8 The DNAR should be activated through Al Shifa using the DNR icon on the top right hand corner.

4.5.9 All health care workers competent to administer CPR shall, when presented with the original or copy of the DNAR order issued by the physician-in-charge that is consistent with this policy, be expected to take appropriate actions to comply with the DNAR order as specified in the form.

4.5.10 Actions contrary to the principles of clause 4.5.9 of this policy, if taken by any health care worker, should be documented in the medical record and the reasons for such actions, as well as inform the physician-in-charge.

4.6 Procedures

4.6.1 Once the DNAR form is complete, it should be handed to the nurse in charge who in turn should keep the form at and in a place where it is easily accessible to the health care workers. The DNAR status should also be documented in the medical record by the physician-in-charge consistent with 4.5.5 of this policy.

4.6.2 The nurse-in-charge of each shift should record the DNAR status of the patient in the nursing records and communicate such decisions during daily hand over.

4.6.3 The DNAR order must be communicated verbally by the physician-in-charge to other health care workers involved in the treatment of the patient.

4.6.4 The DNAR order remains in effect until re-evaluated (4.6.5).

4.6.5 Patients with DNAR order should be seen in ward rounds as any other patients and their DNAR order lifted or modified when their circumstances changes. The decision is to be communicated to the patient or Family / SDM and documented in the medical record as well as in the DNAR form.

with the date and time from when it takes effect.

4.6.7 Electronic archiving of DNAR form may not reflect subsequent reviews and changes and are not amenable to cancellation in case of DNAR withdrawal. Permanent electronic archiving of the signed DNAR form should be done only after cessation of that treatment episode before discharge.

4.6.8 A DNAR order becomes valid when the DNAR form is properly completed, dated, signed and stamped by the physician-in-charge in accordance with clause 4.5.5 and 4.5.6 of this Policy.

4.6.9 If a patient with a DNAR order is transferred to the care of another physician within the same Health Care Facility, the new physician-in-charge should uphold or modify it.

4.6.10 A patient already discharged who had a previous DNAR order may require re-evaluation upon re-admission however in terminally ill patients or in the presence of a disease deemed futile to resuscitate, the DNAR decision remain in effect in any future Health Care Facility admissions.

5. Responsibility for Application:

This policy is to be followed by all the physicians with privileges to practice clinical medicine at their specific Health Care Facilities who are considering a Do-Not-Attempt- Resuscitation order as part of their responsibility in making clinical decisions.

All health care workers who are involved in patient care should be aware of the contents of this policy to aid clinicians in decision making, implementation and to report if there is any violation or misuse of this policy.

In Health Care Facilities that do not have ethical committee, three physicians can replace such committee and pass judgment in controversial decisions.

References

Title of book/ journal/ articles/ Website	Author	Year of publication	Page
“DO NOT RESUSCITATE POLICY”POL 04 REVISION -	SQUH	2011	5-18
Central Bioethics Committee DNAR Documents	SQUH	2016	
The National Bioethics Committee Fifth Forum.Do Not Resuscitate: Searching For An Overall Approach.		2 nd June, 2014	
World Medical Association. Declaration of Helsinki.. Available from: < http://www.wma.net/e/policy/pdf/17c.pdf >.	Ethical Principles for Medical Research Involving Human Subjects	2007	
“Decisions Relating to Cardiopulmonary Resuscitation” 3 rd edition,	British Medical Association, Resuscitation Council (UK) and Royal College of Nursing	October 2014	

resuscitate' orders for the elderly: a survey in Saudi Arabia	Arch Gerontol Geriatr	2000	151-160
Albar MA. Seeking remedy, abstaining from therapy and resuscitation: an islamic perspective..	Saudi J Kidney Dis Transpl	2007	629-637
Association AM. Guidelines for the appropriate use of do-not-resuscitate orders. ;2651868-1871.	Council on Ethical and Judicial Affairs, American Medical Association. JAMA.	1991	1868-1871
Do-not-resuscitate order after 25 years. Crit Care Med	Burns JP, Edwards J, Johnson J, Cassem NH, Truog RD	2003	1543-1550
Increasing use of DNAR orders in the elderly worldwide: whose choice is it? J Med Ethics	Cherniack EP	2002	303-307
Ebell MH. Practical guidelines for do-not-resuscitate orders	Am Fam Physician	1994	1293-1299 1303-1304
Clinical estimation of survival and impact of other prognostic factors on terminally ill cancer patients in Oman. Support Care Cancer.	Faris M	2003	30-34
SUPPORT Investigators. Study to Understand Prognoses and Preferences for Outcomes and Risks of Treatment	Ann Intern Med	1996	284-293
Practicing physicians and the role of family surrogate decision making	Hardart GE, Truog RD	2005	345-354
Attitudes and preferences of intensivists regarding the role of family interests in medical decision making for incompetent patients	Hardart GE, Truog RD	2003	1895-1900
Just how far goes DNAR? J Vasc Surg. Loewy EEH LR. Textbook of Healthcare Ethics. New York: Springer; 2004.	Jones JW, McCullough LB	2008	1630-1632

Postgrad Med J.	Marik PE, Zaloga GP	2001	103-499
CPR in terminally ill patients? Resuscitation.. Emergency Medical Service Association. Guidelines for EMS personnel regarding Do Not	Marik PE, Zaloga GP	2001	103-499
The do-not-resuscitate order: indications on the current practice in Riyadh. Ann Saudi Med.	Mobeireek A	1995	9-156
The practice of do-not- resuscitate orders in the Kingdom of Saudi Arabia. The experience of a tertiary care center. Saudi Med J.	Rahman MU, Arabi Y, Adhami NA, Parker B, Al-Shimemeri A	2004	1278- 1279
Death and dying--a Muslim perspective. J R Soc Med	Sheikh A	1998	138-140
CPR-not- indicated and futility. Ann Intern Med.	Stein RS, Brody H, Tomlinson T, Shaner DM, Shorr AF, Mahmoud RA, et al	1996	75-77
The quality of care plans for patients with do-not- resuscitate orders. Arch Intern Med.	Sulmasy DP, Sood JR, Ury WA	2004	1573- 1578
Recommendations for end-of-life care in the intensive care unit: The Ethics Committee of the Society of Critical Care Medicine. Crit Care Med	Truog RD, Cist AF, Brackett SE, Burns JP, Curley MA, Danis M, et al.	2001	2332- 2348
Resuscitate (DNAR) directives. Available from: www.emsa.ca.gov/default.asp < http://www.emsa.ca.gov/default.asp >.		2007	
Do-Not-Resuscitate Orders. Available from: < http://www.ama-assn.org/go/policyfinder >.		2009	
World Medical Association. Declaration of Helsinki. Ethical Principles for Medical Research Involving Human Subjects. Available from: < http://www.wma.net/e/policy/pdf/17c.pdf >.		2007	

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[Back](#)



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